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QUENVARA (davorixaban) tablets, for oral use

BOXED WARNING

WARNING: (A) PREMATURE DISCONTINUATION OF QUENVARA INCREASES THE RISK OF THROMBOTIC EVENTS; (B) SPINAL/EPIDURAL HEMATOMA

- Premature discontinuation of any oral anticoagulant, including Quenvara, increases the risk of thrombotic events. If Quenvara is discontinued for a reason other than pathological bleeding or completion of a course of therapy, consider coverage with another anticoagulant (5.1).
- Epidural or spinal hematomas may occur in patients treated with Quenvara who are receiving neuraxial anesthesia or undergoing spinal puncture. These hematomas may result in long-term or permanent paralysis (5.3).

1 INDICATIONS AND USAGE

1.1 Nonvalvular atrial fibrillation

Quenvara is indicated to reduce the risk of stroke and systemic embolism in adult patients with nonvalvular atrial fibrillation.

1.2 Treatment of deep vein thrombosis and pulmonary embolism

Quenvara is indicated for the treatment of deep vein thrombosis (DVT) and pulmonary embolism (PE) in adults.

1.3 Reduction in the risk of recurrence of DVT and PE

Quenvara is indicated to reduce the risk of recurrent DVT and PE in adults following at least 6 months of initial treatment.

2 DOSAGE AND ADMINISTRATION

2.1 Recommended dosage

Indication	Recommended dosage
Nonvalvular atrial fibrillation	5 mg orally twice daily
Treatment of DVT or PE	10 mg orally twice daily for the first 7 days, then 5 mg orally twice daily
Reduction in the risk of recurrent DVT or PE	2.5 mg orally twice daily after at least 6 months of treatment for DVT or PE

2.2 Dosage adjustment in nonvalvular atrial fibrillation

The recommended dose is 2.5 mg twice daily in patients who are both 80 years of age or older and weigh 55 kg or less. For dose adjustments in renal impairment, see 2.3. For drug interactions, see 7.1.

2.3 Dosage in renal impairment

Estimate creatinine clearance (CrCl) with the Cockcroft-Gault equation using actual body weight.

Creatinine clearance	Nonvalvular atrial fibrillation	Treatment of DVT or PE	Reduction in risk of recurrence
50 mL/min or more	No adjustment	No adjustment	No adjustment
30 to 49 mL/min	No adjustment	No adjustment	No adjustment
15 to 29 mL/min	2.5 mg twice daily	Not recommended	2.5 mg twice daily
Less than 15 mL/min or on dialysis	Not recommended	Not recommended	Not recommended

Monitor renal function at least every 6 months in patients with CrCl 15 to 49 mL/min, and more often in situations where renal function may decline.

The 2.5 mg twice daily dose for patients with nonvalvular atrial fibrillation and CrCl 15 to 29 mL/min was added in version 5.0 based on the RENEW-Q study (14.4).

2.4 Missed dose

If a dose is missed, the patient should take it as soon as possible on the same day and resume the twice-daily schedule. Do not take two doses at the same time to make up for a missed dose.

2.5 Switching to and from Quenvara

- From warfarin to Quenvara: stop warfarin and start Quenvara when the INR is below 2.0.
- From Quenvara to warfarin: Quenvara affects INR measurements. Stop Quenvara and start warfarin together with a parenteral anticoagulant at the time the next dose of Quenvara would have been taken. Stop the parenteral anticoagulant when the INR reaches an acceptable range.
- From other oral anticoagulants or low molecular weight heparin: start Quenvara at the time the next dose of the other anticoagulant would have been due.

2.6 Temporary interruption for surgery and other interventions

Procedure bleeding risk	Creatinine clearance	Stop Quenvara at least
Low	30 mL/min or more	24 hours before
Low	15 to 29 mL/min	36 hours before
Moderate or high	30 mL/min or more	48 hours before
Moderate or high	15 to 29 mL/min	72 hours before

Restart Quenvara after the procedure as soon as adequate hemostasis is established, usually 24 hours after low bleeding risk procedures and 48 to 72 hours after high bleeding risk procedures. Bridging anticoagulation is generally not required.

2.7 Administration

Swallow Quenvara tablets whole with water, with or without food.

3 DOSAGE FORMS AND STRENGTHS

- 2.5 mg: yellow, round, film-coated tablets debossed with "LQ" on one side and "2.5" on the other.
- 5 mg: pink, oval, film-coated tablets debossed with "LQ" on one side and "5" on the other.

4 CONTRAINDICATIONS

- Active pathological bleeding (5.2).
- Severe hypersensitivity reaction to Quenvara, such as anaphylaxis.

5 WARNINGS AND PRECAUTIONS

5.1 Increased risk of thrombotic events after premature discontinuation

Premature discontinuation of Quenvara in the absence of adequate alternative anticoagulation increases the risk of thrombotic events. An increased rate of stroke was observed after Quenvara was stopped in clinical trials in patients with atrial fibrillation.

5.2 Risk of bleeding

Quenvara increases the risk of bleeding and can cause serious and fatal bleeding. Concomitant use of drugs affecting hemostasis increases the risk of bleeding, including aspirin and other antiplatelet agents, other anticoagulants, heparin, thrombolytic agents, selective serotonin reuptake inhibitors (SSRIs), serotonin norepinephrine reuptake inhibitors (SNRIs) and nonsteroidal anti-inflammatory drugs (NSAIDs).

No specific reversal agent for davorixaban is approved. Activated charcoal reduces absorption of davorixaban if given within 6 hours of the last dose. Hemodialysis does not meaningfully remove davorixaban. Protamine sulfate and vitamin K are not expected to affect its anticoagulant activity.

5.3 Spinal or epidural anesthesia or puncture

When neuraxial anesthesia or spinal puncture is used, patients treated with anticoagulants are at risk of developing an epidural or spinal hematoma. Do not remove an indwelling epidural or intrathecal catheter earlier than 30 hours after the last dose of Quenvara. Do not give

the next dose earlier than 6 hours after the catheter is removed.

5.4 Patients with prosthetic heart valves

The safety and efficacy of Quenvara have not been studied in patients with prosthetic heart valves. Use of Quenvara is not recommended in these patients.

5.5 Patients with triple-positive antiphospholipid syndrome

Direct-acting oral anticoagulants, including Quenvara, are not recommended in patients with triple-positive antiphospholipid syndrome because of an increased rate of recurrent thrombotic events.

6 ADVERSE REACTIONS

6.1 Clinical trials experience

The safety of Quenvara in nonvalvular atrial fibrillation was evaluated in QUELL-AF, in which 9,412 patients were randomized to Quenvara 5 mg twice daily or warfarin and followed for a median of 1.9 years.

Table 2. Bleeding events in QUELL-AF (events per 100 patient-years)

Event	Quenvara (N=4,701)	Warfarin (N=4,711)	Hazard ratio (95% CI)
Major bleeding	2.04	3.11	0.66 (0.57 to 0.76)
Intracranial hemorrhage	0.31	0.79	0.39 (0.28 to 0.55)
Gastrointestinal bleeding	0.84	0.88	0.95 (0.76 to 1.19)
Clinically relevant non-major bleeding	2.71	4.02	0.67 (0.59 to 0.77)

Table 3. Adverse reactions in at least 1% of Quenvara-treated patients in QUELL-AF

Adverse reaction	Quenvara (%)	Warfarin (%)
Epistaxis	4.1	5.3
Contusion (bruising)	3.6	5.2
Hematuria	2.4	3.1
Anemia	2.2	2.0
Gingival bleeding	1.4	2.1
Nausea	1.3	1.1
Rash	1.1	0.9

Discontinuation because of adverse reactions occurred in 6.8% of Quenvara-treated patients and 7.9% of warfarin-treated patients.

7 DRUG INTERACTIONS

7.1 Combined P-gp and strong CYP3A4 inhibitors

For patients receiving Quenvara 5 mg or 10 mg twice daily, reduce the dose by 50% when Quenvara is used with drugs that are combined P-glycoprotein (P-gp) and strong cytochrome P450 3A4 (CYP3A4) inhibitors, such as ketoconazole, itraconazole, ritonavir or clarithromycin. In patients already taking 2.5 mg twice daily, avoid coadministration.

7.2 Combined P-gp and strong CYP3A4 inducers

Avoid concomitant use of Quenvara with combined P-gp and strong CYP3A4 inducers, such as rifampin, carbamazepine, phenytoin or St. John's wort. These drugs reduce davorixaban exposure by about 54%.

7.3 Anticoagulants, antiplatelet agents, NSAIDs, SSRIs and SNRIs

Coadministration increases the risk of bleeding (5.2). Short-term overlap with another anticoagulant is acceptable only when switching (2.5).

8 USE IN SPECIFIC POPULATIONS

8.1 Pregnancy

There are limited data on Quenvara use in pregnant women. Treatment is likely to increase the risk of hemorrhage during pregnancy and delivery. Use only if the potential benefit justifies the potential risk to the mother and fetus.

8.2 Lactation

Davorixaban is present in rat milk. Breastfeeding is not recommended during treatment with Quenvara.

8.4 Pediatric use

Safety and effectiveness in pediatric patients have not been established.

8.5 Geriatric use

Of the patients in QUELL-AF, 38% were 75 years or older. No overall differences in effectiveness were observed. See 2.2 for dose adjustment in patients 80 years or older who weigh 55 kg or less.

8.6 Renal impairment

See 2.3 for dosing by creatinine clearance and 2.6 for interruption before procedures.

8.7 Hepatic impairment

No dose adjustment is required in mild hepatic impairment (Child-Pugh A). Use with caution in moderate hepatic impairment (Child-Pugh B). Quenvara is not recommended in severe hepatic impairment (Child-Pugh C).

10 OVERDOSAGE

Overdose increases the risk of bleeding. Activated charcoal may reduce absorption if given within 6 hours of ingestion. Because of high plasma protein binding, davorixaban is not expected to be dialyzable.

12 CLINICAL PHARMACOLOGY

12.3 Pharmacokinetics

Parameter	Value
Absolute bioavailability	About 62%
Time to peak concentration	3 to 4 hours
Elimination half-life	About 12 hours
Plasma protein binding	About 87%
Excreted unchanged in urine	About 27%
Metabolism	Mainly CYP3A4; substrate of P-gp

Food does not affect the bioavailability of Quenvara.

14 CLINICAL STUDIES

14.1 QUELL-AF

In QUELL-AF, the rate of stroke or systemic embolism was 1.21 per 100 patient-years with Quenvara and 1.54 per 100 patient-years with warfarin (hazard ratio 0.79; 95% CI 0.66 to 0.94).

14.4 RENEW-Q

RENEW-Q randomized 612 patients with nonvalvular atrial fibrillation and CrCl 15 to 29 mL/min to Quenvara 2.5 mg twice daily or warfarin. Stroke or systemic embolism occurred at 2.3 versus 2.6 per 100 patient-years, and major bleeding at 3.9 versus 6.8 per 100 patient-years.

16 HOW SUPPLIED, STORAGE AND HANDLING

Strength	Package	NDC
2.5 mg	Bottle of 60 tablets	99107-201-60
5 mg	Bottle of 60 tablets	99107-205-60
5 mg	Unit-dose blister pack, 10 x 10 tablets	99107-205-11

Store at 20°C to 25°C (68°F to 77°F); excursions permitted between 15°C and 30°C (59°F and 86°F). Keep in the original container.

17 PATIENT COUNSELING INFORMATION

- Do not stop taking Quenvara without talking to the prescriber, because stopping increases the risk of stroke and blood clots.
- Tell all healthcare providers, including dentists, that you take Quenvara before any surgery or procedure.
- Seek medical care for signs of bleeding: unusual bruising, pink or brown urine, red or black stools, coughing up blood, or vomiting blood or material that looks like coffee grounds.

REVISION HISTORY

Version	Effective date	Changes
5.0	2026-03-01	2.3: 2.5 mg twice daily for nonvalvular atrial fibrillation with CrCl 15 to 29 mL/min (RENEW-Q, 14.4); renal monitoring every 6 months. 2.6: interruption times for CrCl 15 to 29 mL/min. 7.1: clarithromycin added.
4.0	2024-06-01	2.2: dose criteria updated. 5.5 added.

Medical Information: 1-800-555-0199, medinfo@larkmoor.example. To report suspected adverse reactions, contact Larkmoor Therapeutics Pharmacovigilance at 1-800-555-0177.

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